

NHS Devon Integrated Care Board Service Specification

Service Specification No.	
Service	General Practice Specialised Medicines Service (Devon Wide)
Commissioner Lead	Nicola Jones, Deputy Director for Primary Care
Provider Lead	
Period	1 st April 2025 to 31 st March 2029 (3 months' notice will be required to terminate this contractual agreement by either party)
Date of Review	Annual review re inflation Formal LES review: October 2028

1. Population Needs

1.1 National/local context and evidence base

The changes brought about by the Health and Social Care Act 2012 necessitated the re-specification of the payment schemes previously undertaken under enhanced services to the primary care medical contract.

Previous discussions between the ICB (formerly CCGs) and the LMC indicated a unified framework across Devon for schemes involving drugs would be desirable. Building on these discussions, the enhanced services framework which had been previously agreed in principle was further developed and validated against a wider range of medication-related enhanced services. This framework has found to have a high degree of face validity. That is, drugs for which more practice level input is required or where the impact on a GP's workload is greater produce higher scores.

The enhanced services framework was developed to provide a means of assessing the impact of a drug on primary care activity and determine the value of this work in a transparent and consistent manner. It was envisaged that the framework would be used to fund the INR monitoring service, shared care guidelines, the prescribing and administration of GnRH analogues (e.g. Zoladex®) and additional work associated with some specialised drugs prescribed in primary care following secondary care initiation which may be considered to be beyond the scope of essential services outlined in the GMS contract. The framework has been accepted by the LMC for the general practice specialised medicines service since 2014-2015.

2. Outcomes

2.1 NHS Outcomes Framework Domains & Indicators

Domain 1	Preventing people from dying prematurely	✓
Domain 2	Enhancing quality of life for people with long-term conditions	✓
Domain 3	Helping people to recover from episodes of ill-health or following injury	✗
Domain 4	Ensuring people have a positive experience of care	✓
Domain 5	Treating and caring for people in safe environment and protecting them from avoidable harm	✓

2.2 Local Defined Outcome

The agreement of this General Practice Specialist Medicines Service would permit the equitable funding of specialised medication activity across all areas of Devon, on the basis that where the ICB agrees funding for primary care in one geographical location in Devon the fee will be commensurate with similar activity elsewhere in Devon. The scheme will permit local implementation of initiatives to meet local needs where funding is made available, without prejudice to other areas with different needs or where funding is not available.

It is proposed that the current amount of funding for this Service is distributed over the funded schemes by means of a five-tiered payment structure using this framework.

3. Scope

3.1 Aims and objectives of service

The aim of the proposed Specialist Medicines Service is to permit equitable funding of specialised medication activity across all areas of Devon through use of a tiered framework which incorporates staff activity, impact of lack of familiarity with a drug on a GP's workload and clinical responsibility.

GPs taking on responsibilities through this service framework do so on a case-by-case basis as has always been the case with "shared care".

3.2 Service description/care pathway

The framework below incorporates staff activity including administration and visits to a healthcare assistant, nurse or GP, interpretation of diagnostic tests and impact of lack of familiarity of a drug on a GP's workload and clinical responsibilities. The framework covers the cost of patient care. The costs of using practice owned equipment, as is the case with near patient INR testing, is not included in the framework and would be an additional payment to be overall commensurate with the former anticoagulation NES.

The framework is shown below and accompanied by assumptions used in assigning weights to the various medicine's services currently funded.

			Score					Weighting
			0	1	2	3	4+	
5	Admin Input	Not required		Single process	on-going recall system or single process with follow up	on-going recall system with follow up	Exceptional	7.5

		Routine HCA Input	Not required	4 or less contacts pa.	5-7 contacts pa	8 - 12 contacts pa	Exceptional	7.5
		Routine nurse Input	Not required	4 or less contacts pa.	5-7 contacts pa	8 - 12 contacts pa	Exceptional	15
		Routine GP Input	Not required	4 or less contacts pa.	5-7 contacts pa	8 - 12 contacts pa	Exceptional	50
		Diagnostic Tests	Not required	HCA interpretation	Nurse interpretation	routine GP interpretation	Exceptional	5
	Impact assessment	Impact of unfamiliarity with drug	Established drug used in > 1 in 500 pts	New drug used in > 1 in 500 pts	use in between 1 in 500 and 1 in 2000 pts	use in fewer than 1 in 2000 pts		5
		Governance/ Clinical responsibility	Not applicable	Full for established drug (use in conjunction with impact score 0)	Shared	Full for less familiar drug (use in conjunction with impact score of 1,2 or 3)		10

The assumptions used in testing the framework are listed below.

Administrative input: it is assumed that an on-going recall system with follow-up would be used for existing guidance.

Routine HCA (health care assistant) input: used when blood samples taken, or anthropometric measurements required.

Routine nurse input: used when clinical measurements required (e.g. blood pressure or pulse). If blood sample also required, it is assumed that the nurse would take this at the same visit. Also, used for administration of injections.

Routine GP visit: requires patient consultation for a specified reason documented in guidance above normal level of care for patient with this diagnosis.

Diagnostic tests: it is assumed that all results of blood tests, clinical and anthropometric measurements are interpreted by GP. There is no interpretation of tests or measurements leading to a clinical decision by a health care assistant or nurse.

Impact of unfamiliarity of drug: this category allows for the additional impact on a GP's workload of care for a patient receiving a drug which is not routinely used in primary care. This allows for new drugs and for established drugs which are not frequently prescribed.

Governance / clinical responsibility: the score for this category is conditional on the familiarity of the GP with the drug. Clinical responsibility is always assigned a weighting, whether it is shared responsibility or full clinical responsibility. Shared care and taking full clinical responsibility for drugs which are new or not frequently prescribed in primary care is given a higher score than full clinical responsibility for drugs routinely prescribed in primary care.

Outcome of testing framework

- The scores achieved demonstrate that the framework differentiates between level of activity, both in terms of frequency of visits to the practice and the type of staff involved, and in terms of the impact on a GP's workload of prescribing new drugs and drugs which are not frequently used in primary care.
- The scores increase with an increase in level of activity. The lowest scores are achieved for guidelines where the drug is prescribed by the GP and all other care is undertaken by secondary care.
- The highest scores are achieved for guidelines where frequent visits to a nurse are required for drug administration or clinical measurements (e.g. regular monitoring of blood pressure and/or pulse) together with the taking of blood samples and the interpretation of test results by a GP.
- The impact of prescribing new drugs or established drugs which are not frequently prescribed in primary care is reflected by the scores at every level of activity. Higher scores are achieved for drugs which are not frequently prescribed in primary care, for shared care and for taking on full responsibility for less familiar drugs.

Developing the tiers to determine funding

To determine the number of tiers and point threshold of each tier, all possible levels of activity were replicated for each combination of the two variables, impact of unfamiliarity with a drug and level of clinical responsibility. Using the framework, 29 levels of activity were identified which might feasibly occur in clinical practice. This included levels of activity outside the scope of the current shared care guidelines and LES for INR to future proof the framework.

Eight combinations of familiarity with the drug and level of clinical responsibility were identified which when combined with levels of activity gave a total of 232 clinical scenarios. Similar levels of activity were grouped together resulting in five groups, with the highest group representing exceptional circumstances. Thus, five funding tiers were adopted.

The point threshold for each tier was calculated from the range of points from the lowest point of the first tier to the lowest point of the fifth tier. Figure 1 (Appendix 1) shows the allocation of tiers across the clinical scenarios.

The level of funding increases as the level of activity increases and as the impact of unfamiliarity of a drug and level of clinical responsibility increases. Shared care guidelines funded by NHS Devon and NHS Plymouth and currently funded by NHS Devon ICB are included in Figure 1 (Appendix 1) as are levels 1 and 2 of the INR service.

Payment for each tier

For planning the new framework, the amount of money available was based on historical spend (i.e. the

total amount paid out next year would be the same if activity remained the same but there is a redistribution of how this is paid out).

Tier 1: Fee per patient is £62.22

- Equivalent to highest locally agreed cost for drugs which fall into tier 1, that is GnRH analogues.
- Western locality practices receive funding at same level as currently.
- Payments for Eastern and Northern locality practices will increase to bring them in line with Western locality practices.

Tier 2: Fee per patient is £105.94

The cost for tiers 2 and 3 has been determined by calculating the mean payment per point based on the current funding of drugs within a specific tier and multiplying this figure by the mean point score for the same drugs.

- Pays higher for some currently funded drugs and lower for other drugs.
- This tier includes drugs which are funded in some areas and not others. Therefore practices in some areas will be funded for work which they were not previously funded for.
- Money from this tier will also be used to fund the highest paid tier.

Tier 3: Fee per patient is £132.42

- Payment for tier 3 is equivalent or higher than currently funded cost of shared care for drugs which fall into this tier.

Tier 4: Fee per patient is £158.90

- Tier 4 payment is approximately 30% higher than the current shared care payment for drugs within this tier in recognition of the complexity of work associated with these drugs.
- This tier includes drugs which are funded in some areas of the ICB but not others. Therefore, practices in some areas will be funded for work which they were not previously funded for.

The fifth tier is for drugs judged to represent an exceptional workload. None of the current shared care guidelines or LES for medicines falls into this tier.

DDRB will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.

In the event of a pandemic or other 'level 4 health' event, commissioner and LMC will meet urgently to review this specification giving consideration to:

- a. suspension of some or all elements of it,
- b. redirection of patients to alternative provider(s),
- c. in the event of actioning either a) or b) determining appropriate income protection arrangements for GP provider

Listed below are all the drugs included within the SMS LES. Please refer to the list of SMS guidelines included here (<https://onedevon.org.uk/our-work/services-and-support/medicines-and->

[treatments/clinical-guidance-for-prescribers/](#)) for therapeutic areas and indications covered.

The SMS guidelines can also be accessed via the Devon Formulary (<https://devonformularyguidance.nhs.uk/>) under the individual drug entries.

Medicine	Funding tier (per patient, per year)	Locality applicable to			
		Northern	Eastern	Western	Southern
Atomoxetine	2	Yes	Yes	Yes	Yes
Amiodarone <i>(Added 1st Oct 25)</i>	2	Yes	Yes	Yes	Yes
Amisulpride (Claim from 1 st April 26)	2	Yes	Yes	Yes	Yes
Aripiprazole – oral formulation (Added 14 th Sept)	0	Yes	Yes	Yes	Yes
Azathioprine	2	Yes	Yes	Yes	Yes
Ciclosporin	4	Yes	Yes	Yes	Yes
Dapsone <i>(Devon wide from 28th Oct 19)</i>	2	Yes	Yes	Yes	Yes
*Denosumab for Osteoporosis	2(Year 1)	Yes	Yes	Yes	Yes
*Denosumab for Osteoporosis	3 (Year 2 onwards)	Yes	Yes	Yes	Yes
Dexamfetamine – <i>(Added May 2023)</i> <i>(Added Jun 2023)</i>	2	Yes (from 1 May 23)	Yes (from 1 May 23)	Yes (from 1 May 23)	Yes
Flupentixol decanoate depot injection <i>(From 1st March 2020 for DPT patients only)</i>	4	Yes	Yes	Yes (excludes Plymouth practices (only available to DPT facing prs))	Yes
Gonadorelin analogues - Prostate	1	Yes	Yes	Yes	No ***
Guanfacine <i>(Added March 2024)</i>	2	Yes	Yes	Yes	Yes
Haloperidol – oral formulation (Claim from 1 st April 26)	2	Yes	Yes	Yes	Yes
Haloperidol decanoate depot injection <i>(From 1st March 2020 for DPT patients only)</i>	4	Yes	Yes	Yes (excludes Plymouth practices (only available to DPT facing prs))	Yes
Hydroxycarbamide	2	Yes	Yes	No	No
Leflunomide	3	Yes	Yes	Yes	Yes
Lisdexamfetamine	2	Yes	Yes	Yes	Yes
Lithium	1	Yes	Yes	Yes	Yes
Mercaptopurine <i>(added 1st May 2022)</i>	2	Yes	Yes	Yes	Yes
Methotrexate - oral formulation only	2	Yes	Yes	Yes	Yes
Methotrexate – subcutaneous (Dermatology)	2	No	No	Yes	No

Methotrexate – subcutaneous (Rheumatology)	2	Yes	Yes	Yes	No
Methylphenidate	2	Yes	Yes	Yes	Yes
Modafinil	2	Yes	Yes	Yes	Yes
Mycophenolate mofetil (Rheumatology)	3	Yes	Yes	Yes	No
Mycophenolate mofetil and mycophenolic acid (Dermatology, ophthalmology & neurology) <i>(added 1st Apr 2024)</i>	3	Yes	Yes	Yes	Yes
Olanzapine – oral formulation (Claim from 1st April 26)	2	Yes	Yes	Yes	Yes
Penicillamine - <u>claims for this drug should be for existing patients only</u>	3	Yes	Yes	No	No
Quetiapine (Claim from 1 st April 26)	2	Yes	Yes	Yes	Yes
Riluzole	2	Yes	Yes	Yes	Yes
Risperidone – oral formulation (Claim from 1 st April 26)	2	Yes	Yes	Yes	Yes
Sodium oxybate	2	Yes	Yes	Yes	No
Solriamfetol <i>(Devon wide from May 2023)</i>	2	Yes	Yes	Yes	Yes
Sulfasalazine	1	Yes	Yes	Yes	Yes
**Warfarin	2	Yes	Yes	Yes	Yes
Zuclopenthixol decanoate depot injection <i>(From 1st March 2020 for DPT patients only)</i>	4	Yes	Yes	Yes (excludes Plymouth practices (only available to DPT facing pts))	Yes

* **Denosumab:** the fee is lower for the first year of treatment to reflect specialist initiation of treatment. From the second year of treatment onwards, primary care is expected to take full responsibility for patients receiving denosumab.

** **Warfarin:** INR service specification levels 1 and 2:

Level 1: For practice-funded phlebotomist or pharmacist etc, practice sample, laboratory test, practice dosing and resource required. The level 1 service will receive a tier 2 fee, plus an additional enhanced payment of £3.29 to reflect the resource required for warfarin services within general practice.

Level 2: For practice-funded phlebotomist or pharmacist etc, practice sample, practice test, practice dosing, resource required and to cover the costs of consumables and equipment *(excluding testing strips which are funded by the ICB direct to the supplier). The level 2 service will receive a tier 2 fee, plus an additional fee £47.80 to reflect the resource required and cover the costs of consumables and equipment (excluding testing strips which are funded by the ICB direct to the supplier).

*** Administration of Gonadorelin Analogues for male patients (Prostate Cancer) LES available for SDT area

Periodic review

The LMC will be consulted about the drugs to be funded under this scheme on a periodic basis. These discussions will allow for recognition of the impact of new specialised medicines and that some specialised medicines will have become established in practice and familiar to GPs.

3.3 Population covered

The registered population of the practice

3.4 Any acceptance and exclusion criteria and thresholds

It is envisaged that the framework will be used to fund the INR monitoring service, shared care guidelines, the prescribing and administration of GnRH analogues (e.g. Zoladex®) and additional work associated with some specialised drugs prescribed in primary care following secondary care initiation which may be beyond the scope of essential services outlined in the GMS contract.

3.5 Interdependence with other services/providers

Where a future shared care guideline or service redesign is proposed the framework will be used to show the proponents what the primary care commissioning service cost will be. This allows identification of the funding that is needed for the service design to be explicitly known early in the process.

4. Applicable Service Standards

Providers and practitioners will adhere to the local and national guidelines or any updated versions thereof.

4.1 Applicable national standards (e.g., NICE)

4.2 Applicable standards set out in Guidance and/or issued by a competent body (e.g. Royal Colleges)

4.3 Applicable local standards

This document presents an overarching proposal for primary care payment for specialised medicines. The services and shared care guidelines covered by this payment scheme have each been developed taking into account applicable national standards (e.g., NICE guidance), guidance from professional bodies and applicable local standards.

5. Applicable quality requirements and CQUIN goals

5.1 Applicable Quality Requirements (See Schedule 4 Parts [A-D])

Not applicable.

5.2 Applicable CQUIN goals (See Schedule 4 Part [E])

Not applicable.

6. Location of Provider Premises

Main surgery and any branch sites as per GMS, PMS or APMS agreement.

7. Individual Service User Placement

Not applicable

Appendix 1

Table 1: Results of testing specialised medicines payment framework on existing shared care guidelines for North and East Devon, Torbay and Plymouth and LES for INR monitoring

Level of activity for GP practice	Weighted Score	Comment
All care undertaken by secondary care		
	0	Level 1 of former anticoagulation NES
Prescribing		
	25-35	Range of scores reflects familiarity with drug. Highest scores given to drugs estimated to be prescribed in fewer than 1 in 2000 patients
Prescribing and administration		
Monthly administration	102.5	Injection - scores reflect administration by nurse and high score for lack of familiarity
Three monthly administration	67.5	Injection - administered by nurse. More frequently prescribed drug than previous example
Prescribing and taking bloods		
Three monthly monitoring	65	High score for lack of familiarity
Taking bloods and interpreting results		
Monthly monitoring	80	Established drug
Taking bloods, BP and interpreting results		
Two monthly monitoring	102.5	High score for lack of familiarity
Prescribing, taking bloods and interpreting results		
Monthly monitoring	70-95	Lowest score for frequently used established drugs. Higher scores reflect shared care and lack of familiarity with drug.
Two monthly monitoring	87.5	Higher score for lack of familiarity
Three monthly monitoring	65-85	Higher scores reflect lack of familiarity with drug and level of responsibility ranging from shared care to full responsibility for a less frequently prescribed drug
Six monthly monitoring	80	Higher score for lack of familiarity
Prescribing, taking bloods, BP, interpreting results		
Monthly monitoring	102.5 -	Higher score reflects difference between established drug and drug estimated to be prescribed in less than 1 in 2000 patients
Three monthly monitoring	87.5	High score for lack of familiarity
Six monthly monitoring	77.5	New drugs estimated to be used in more than 1 in 500 patients
Prescribing, administration of drug, taking bloods and interpreting results		
Monthly administration and monitoring	117.5	High score for lack of familiarity

Figure 1: Level of activity and tier for determining funding

	Impact of unfamiliarity with drug and degree of clinical responsibility*							
	Established , full, >1 in 500	Established , shared care, >1 in 500	New drug, shared care, >1 in 500	Between 1 in 500 & 1 in 2000, shared	<1 in 2000, shared	New drug, full, >1 in 500	Between 1 in 500 & 1 in 2000, full	<1 in 2000, full
Activity level								
Prescribing only of agreed drugs								
HCA taking bloods or measuring weight ≤ 3 monthly.	N/A					N/A	N/A	N/A
HCA taking bloods or measuring weight approx. 2 monthly.	N/A					N/A	N/A	N/A
Administration and/or clinical measurements by nurse ≤ 3 monthly				GnRH				
HCA taking bloods or weighing every 4 to 6 weeks	N/A					N/A	N/A	N/A
HCA taking bloods or measuring weight ≤ 3 monthly. GP interpretation of test results		Sulfa			Rilozole			
HCA taking bloods or measuring weight approx. 2 monthly. GP interpretation of test results								
Administration and/or clinical measurements by nurse approx. two monthly								
Administration and/or clinical measurements by nurse ≤ 3 monthly. GP interpretation			Atomox, Lisdex, Methlyph en					
HCA taking bloods and nurse administration ≤ 3 monthly. GP interpretation of tests	N/A			Denosumab for osteoporosis (Year 1)		N/A	N/A	N/A
HCA taking bloods or weighing every 4 to 6 weeks. GP interpretation of test results.	INR level 1 and 2	AZA. MTX			MTP, pencilla			

Administration and/or clinical measurements by nurse every 4 to 6 weeks								
HCA taking bloods and nurse administration <=3 monthly. GP interpretation of results.							Denosumab for osteoporosis (Year 2 onwards)	
HCA taking bloods > monthly. GP interpretation of test results								
GP consultation ≤ 3 monthly								
GP consultation and HCA visit (eg. taking bloods) ≤ 3 monthly								
Administration and/or clinical measurements by nurse every 4 to 6 weeks. GP interpretation of results		Leflunomide			Ciclo			
Administration and/or clinical measurements by nurse > monthly								
GP consultation and nurse visit ≤ 3 monthly								
Administration and/or clinical measurements by nurse > monthly. GP interpretation of test results								

Notes:

Tier 5 represents exceptional levels of activity and includes activities where frequent consultation with a GP is required in addition to the usual level of care for a patient with a specific diagnosis.

Shared care guidelines currently funded by NHS Devon and level 1 and 2 INR services are included in Figure One.

Key:

Atomox: Atomoxetine

AZA: Azathioprine

Ciclo: Ciclosporin

GnRH: Gonadorelin analogues

Lisdex: Lisdexamfetamine

Methylphen: Methylphenidate

MTP: Mercaptopurine for inflammatory bowel disease

MTX: Methotrexate

Penicilla: Penicillamine

Sulfa: Sulfasalazine

N/A: clinical scenario not applicable to drugs where the GP takes on full clinical responsibility. Under full clinical responsibility, the results of blood samples or anthropometric measurements taken by an HCA would be interpreted by the GP.

*Impact of unfamiliarity with drug and degree of clinical responsibility

Unfamiliarity with drug:

- Established drug used in >1 in 500 patients
- New drug used in >1 in 500 patients
- Use in between 1 in 500 patients and 1 in 2000 patients
- Use in fewer than 1 in 2000 patients

Clinical responsibility:

- “full” responsibility or “shared care”